Childbearing Women and Newborns

Tips for Childbearing Women

- Usually, pregnancy is not an emergency. In fact, if the pregnant woman is otherwise healthy, it's likely that she can be included in any plans for evacuation or sheltering for the general population.
- However, if the woman has had a cesarean section ("C-Section") at any time in the past, or if she has any of the following problems now or in the previous three hours, she is at higher risk.
  - Steady bleeding "like a period" from the vagina
  - Convulsion or a really bad (unusual) headache that will not go away with Tylenol
  - Constant strong belly or back pain with hardness in her pregnant belly
  - Strong pains and hardening belly that comes and goes every couple of minutes and a "due date" three weeks away or more
    - If she has had any of these problems, she should be taken to a hospital (if hospital access is available) or other health care facility for an assessment. If taking her to a facility is not possible, she should be helped to find a comfortable position and not be left alone.

- If she has not had these problems, the hospital is often not the best place to take pregnant women, women in labor or new mothers with newborns due to danger from infections or other exposures. Remember: a normal birth is not an illness.

- A woman who has one or more of the symptoms below may be in labor and about to give birth. Do not move her - it is better to have a birth where you are than on the way to somewhere else.
  - Making grunting sounds every one to three minutes.
  - She says "yes" if you ask "Is the baby pushing down?" or she says, "The baby's coming."
  - You see bulging out around the vagina when she grunts or bears down.
- Give pregnant women and new moms lots of fluids to drink (water or juice is best).
- Be as calming as possible; expectant mothers may be especially anxious in emergency situations. Reassure them you will do everything you can for them.
- Try not to separate expectant or newly delivered moms and their family, even if transporting.

- If you must transport a pregnant woman, regardless of whether she is in labor or not:
  - Transport her lying on her side, not flat on her back
  - Ask her if she has a copy of her pregnancy/prenatal records; if she does, make sure they are brought with her.

Tips for Just After a Baby has Been Born

- Dry and rub the baby gently to keep baby warm and to stimulate breathing.
- Place the naked baby on mother's skin between her breasts and cover both mom and baby.
- Cutting the cord is not an emergency. The cord should only be cut when you have sterile tools (scissor, knife blade, etc.). It's better to wait rather than cut the cord with a non-sterile blade.
- Usually, the placenta (afterbirth) will follow the baby on its own in about a half an hour or less. After it comes, it can be put in a plastic bag, wrapped with the baby or left behind, depending on the circumstances.

- Monitor bleeding from the vagina. Some bleeding is normal - like a heavy period. It should slow down to a trickle within 5 to 10 minutes. If it doesn't, the woman needs medical care.
- Encourage mom to put baby to breast. The baby's hands should be free to help find the breast. Point baby's nose toward mom's nipple and the baby's tummy toward mom's.
- If you need to transport a mother and her newly born baby:
  - Keep the mom and newborn together: baby in mom's arms or on her belly
  - Take diapers, baby clothes and formula and bottles (if mom is bottle feeding the baby) if they are available.